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Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°170/MVE-BDBV/31/10/2026

Reporting date: October 31, 2026 Publication date: November 1, 2026

HIGHLIGHTS (last 24 hours)

For October 31, 2026, fifty-three (53) new confirmed cases and 45 deaths (26 community deaths and 19 in Ebola Treatment Centers) of Ebola Virus Disease (EVD) were reported in the provinces of Ituri (32), North Kivu (18), and Haut-Uélé (3). A total of 421 new samples were analyzed, and 53 of these samples tested positive for Bundibugyo virus, representing a positivity rate of 12.6%. No new health zones were affected in the last 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10467	5103	48.8%	980	2,824	86.6%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

67 out of 178 health zones affected (37.6%)

Summary of key points from the last 24 hours

At the end of the day on October 31, 2026, **fifty-three (53) new confirmed cases** Cases of Ebola virus disease (EVD) have been reported in three provinces: Ituri (32 cases), North Kivu (18 cases), and Haut-Uélé (3 cases). Among these cases, **26 community deaths** have been reported (13 in Ituri, 12 in North Kivu, and 1 in Haut-Uélé). In addition, **19 deaths have been confirmed.** were recorded in the CTEs (15 in Ituri, 4), bringing to **45** the total number of confirmed deaths notified during the period.

In addition, **21 patients** (12 in Ituri, 8 in North Kivu and 1 in Haut-Uélé) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded **10,467 confirmed cases, including 5,103 deaths, representing an overall case fatality rate of 48.8** %. Ituri remains the epicenter of the epidemic, accounting for 70.8% of cumulative confirmed cases and 60.4% of newly reported cases. North Kivu represents 34.0% of new cases and has a case fatality rate of 55.7%, higher than the national average.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 31, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	32	7412	3,483	47.0%	28/36 (77.8%)
North Kivu	18	2479	1,382	55.7%	17/34 (50.0%)
Haut-Uélé	3	460	204	44.3%	7/13 (53.8%)
Tshopo	0	92	27	29.3%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	4/11 (36.4%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	0	6	1	16.7%	2/11 (18.2%)
Total	**53**	**10,467**	**5,103**	**48.8%**	**67/178 (37.6%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 31, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7412**	**3483**	**47.0%**	**32**	**13**	**15**	**28**
Adja	11	1	9.1%				0
Aru	25	7	28.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	220	49	22.3%				0
Boga	2	2	100.0%				0
Bunia	2007	699	34.8%	8	5	2	7
Damascus	41	10	24.4%				0
Drodro	15	7	46.7%				0
Fataki	84	31	36.9%				0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	40	13	32.5%				0
Komanda	193	118	61.1%				0
Lita	329	165	50.2%	2	0	0	0
Logo	14	9	64.3%				0
Lolwa	37	11	29.7%			1	1
Mahagi	5	1	20.0%				0
Mambasa	101	53	52.5%	5	4	2	6
Mandima	488	262	53.7%	9	3	5	8
Mangala	398	172	43.2%	1	0	0	0
Mongbwalu	722	348	48.2%	2	1	0	1
Nia-Nia	341	197	57.8%				0
Nizi	853	312	36.6%	2	0	2	2
Nyankunde	139	43	30.9%				0

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Rimba	25	6	24.0%	2	0	0	0
Rwampara	1203	433	36.0%	1	0	3	3
Tchomia	86	27	31.4%				0
To ventilate	N / A	493	N / A				0
North Kivu	**2479**	**1382**	**55.7%**	**18**	**12**	**4**	**16**
Blessed	703	467	66.4%	13	11	1	12
Good	26	9	34.6%				0
Butembo	343	200	58.3%	1	0	1	1
Goma	1	0	0.0%				0
Kalunguta	97	44	45.4%	1	1	0	1
Katwa	677	415	61.3%	1	0	2	2
Kayna	3	2	66.7%				0
Kyondo	46	22	47.8%				0
Lubero	43	15	34.9%				0
Mabalako	116	39	33.6%				0
Alimbongo	6	4	66.7%				0
Manguredjipa	16	5	31.3%				0
Masereka	41	13	31.7%				0
Musienene	161	77	47.8%				0
Mutwanga	55	14	25.5%				0
Oicha	89	38	42.7%	2	0	0	0
Vuhovi	56	18	32.1%				0
Upper Uele	**460**	**204**	**44.3%**	**3**	**1**	**0**	**1**
Boma Mangbetu	46	19	41.3%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	114	53	46.5%	1	0	0	0
Pawa	113	65	57.5%				0
Rungu	7	4	57.1%				0
Wamba	176	61	34.7%	2	1	0	1
Tshopo	**92**	**27**	**29.3%**	0	0	0	0
Bafwasende	18	3	16.7%				0
Kabondo	20	2	10.0%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	37	14	37.8%				0
Mangobo	10	7	70.0%				0
Tshopo	5	0	0.0%				0
Wanie - Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**6**	**1**	**16.7%**	0	0	0	0
Wapinda	3	1	33.3%				0
Yakoma	3	0	0.0%				0
Total	**10467**	**5103**	**48.8%**	**53**	**26**	**19**	**45**

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* An exercise to harmonize non-inventory data was carried out on October 29, 2026 between the Data Management pillar and the SGI and the provincial of Ituri. This exercise led to adjustments in the data reported in the province.

Table 3. Status of alerts notified by province for October 31, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated ed	Number of suspects No investigated ed	Number of suspects investigated and transferred At CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased			
Haut-Uélé	75	5	80	46	5	29	0	51	0	12
Bas Uélé	172	1	173	16	1	156	0	17	0	0
Ituri	1,324	82	1,406	139	82	1149	0	210	11	112
North Kivu	1,423	64	1,487	147	64	1,276	0	210	1	95
South Ubangi ND		ND	ND	ND	ND	ND	ND	ND	ND	ND
North Ubangi ND 99		ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo		1	100	5	1	94	0	6	0	1
Total General	**3,093**	**153**	**3,246**	**353**	**153**	**2,704**	**0**	**494**	**12**	**220**

Figure 1. Distribution of confirmed Ebola virus disease cases by date of symptom onset and outcome from April 20 to October 19

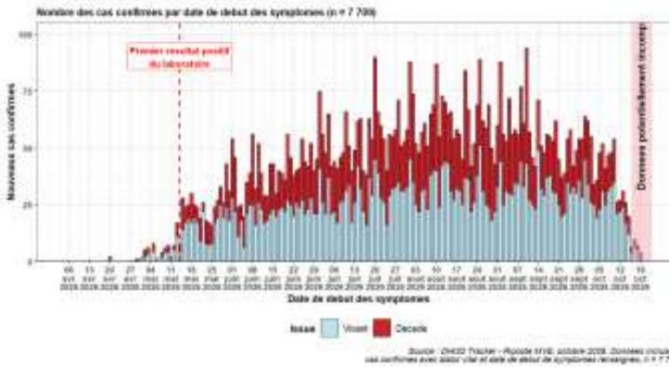
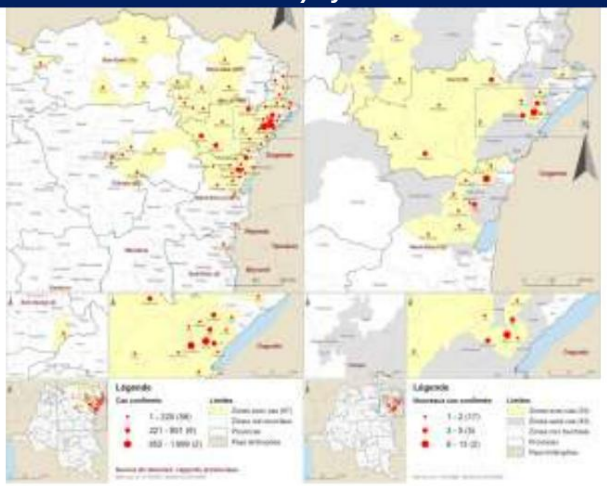


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars; closing in Kisangani, on October 31, 2026, of the work of the Intra-Action Review (IAR) of the response against the 17th Ebola outbreak (Day 3), with systematic screening and hand hygiene of approximately 250 participants and a PSEA briefing at the end of which 190 out of 200 participants (95.0%) signed the code of good conduct.

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2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.2. Main Actions

As of October 31 , 2026, **3,246 alerts** were notified in the five provinces that transmitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu and Tshopo), including **3,093** alerts of living people and **153 deaths ;** 3,209 were verified (98.9%). Of these, 506 (15.6%) were validated as suspected cases and 2,704 (83.3%) were invalidated. Of the 506 validated suspected cases, 494 (97.6%) were investigated and 220 were transferred to CT/CTE

In total, **36,517** contacts were under follow-up in the six provinces that submitted their data ; **31,621** were seen in the last 24 hours, representing a follow-up rate of 86.6%, below the required threshold of 95.0% but an improvement compared to the previous day (85.8%). By province, the follow-up rate is 100% in Bas-Uélé (695/695), 94.3% in Ituri (16,133/17,106), 89.3% in Sud-Ubangi (109/122), 79.4% in Haut-Uélé (1,682/2,119, of which only 68.8% are in Boma Mangbetu and Wamba), 79.3% in Nord-Kivu (12,314/15,533, of which 66.2% are in Beni and 61.9% in Kalunguta) and 73.0% in Tshopo (688/942), where the health staff strike and attacks against teams continue to disrupt follow-up (Table 4).

Table 4. Status of contact tracing by province, as of October 31, 2026

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	17,106	16,133	94.3%
Haut-Uélé	2,119	1,682	79.4%
North Kivu	15,533	12,314	79.3%
Tshopo	942	688	73.0%
Bas-Uélé	695	695	100.0%
South Ubangi	122	109	89.3%
North Ubangi	N / A	N / A	N / A
Total	36,517	31,621	86.6%

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For the day of October 31, eight (8) alerts were notified: 4 in Ituri, 3 in South Kivu and 1 in North Kivu.
The validated alerts have been transferred to the CTEs.

Table 5. Status of key PoE/PoC indicators by province, as of October 31, 2026

Indicators	Ituri	North-Kivu	South Kivu	Tshopo	Bas-Uélé	Overall
Completeness of the of reports	94% (58/62)	95% (20/21)	56% (5/9)	100% (18/18)	64% (14/22)	87.1% (115/132)
Strategic PoC/PoE	1	N / A	N / A	N / A	N / A	1
Number of past travelers	145,370	92,000	7,150		21,766	6,485	272,771
Proportion of travelers screened	97%	98%	99%	99%	98%	98%

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% of travelers who washed their hands	97%	98%	99%	98%	96%	97%
% of travelers made aware	99%	98%	100%	100%	74%	98%
Number alerts notified	04	01	03	00	00	8
Number of validated live alerts	03	01	00	00	00	4
Number of lifeless bodies intercepted	01	00	03	00	00	4
% of suspected cases transferred to CT/CTE	100% (3/3)	100% (1/1)	0%	0%	0%	100% (4/4)
% of lifeless bodies swabed/ secured	0% (0/1)	0%	0% (0/3)	0%	0%	0% (0/4)
% of suspected cases investigated within 24 hours	100% (3/3)	100% (1/1)	0%	0%	0%	100% (4/4)

2.2.2. Challenges

In Ituri, the service providers at the Mongbwalu and Mandima PoCs still need training, there is a lack of mobility for supervision in Bunia and Rwampara, September salaries have not been paid, and the Mandima health zone does not have an ambulance to serve its PoCs; **in North Kivu,** the Butuhe PoC remains closed since the security incident of September 26, 2026, and 1,826 travelers (1.98%) refused to wash their hands; **in South Kivu,** completeness remains low (56%; 5/9), the Baraka-Fizi post's outposts are not reporting, the Kavimvira, Kalundu Port, and Baraka PoEs do not have isolation facilities, and none of the 3 intercepted bodies have been swabé; **In Bas-Uélé,** completeness remains low (64%; 14/22) and providers of PoC PK12 Isiro and PK25 Koteli are threatening to strike for non-payment; **in Tshopo,** 0.9% of travellers refused screening and 1.7% refused handwashing.

2.3. Laboratory

2.3.1. Main actions

- **Ituri:** 32 new positive results (19 living and 13 dead) out of 216 new samples analyzed (positivity: 14.8%).
- **North Kivu:** 18 new positive results (6 living and 12 dead) retained by surveillance; the laboratories of Beni and Butembo analyzed 167 new samples (positivity: 10.8%) and 38 follow-up samples.
- **Tshopo:** 9 samples from living people were analyzed, all negative (positivity: 0%).
- **Haut-Uélé:** 3 new positive results (2 living and 1 death) out of 11 new samples analyzed at Isiro laboratory (positivity: 27.3%).
- **Bas Uélé:** 18 samples from living people were analyzed, all negative (positivity: 0%).

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 27 rings open out of 33 (81.8%), 74 households decontaminated out of 74 (100%) and 11 FOSA out of 13 (84.6%); 66 EDS carried out out of 91 alerts, including 10 postponements (72.5%), 59 bodies swabed out of 78 and 12 EDS postponed to the following day; the CREC facilitated the decontamination of 53 households out of 53, including 2 structures, and 3 vehicles.
- **North Kivu:** 22 rings opened out of 26 (84.6%), 12 households decontaminated out of 16 (75.0%) and 12 ESS out of 13 (92.3%); training and capacity building: 420 service providers briefed, 164 follow-ups and support of ESS and 7 scorecard assessments in 7 ESS (average score: 50.5%); 4 ESS equipped with PCI kits (Katwa and Kyondo), 303 DLM functionality follow-ups and 6 triage units installed; 2 schools decontaminated and 4 assessed in Beni; 58 EDS carried out out of 79 alerts, reports and 59 bodies swabed.
- **Tshopo:** decontamination of the Maleke school complex around the 83rd confirmed case (Kabondo Health Zone); briefing of 121 service providers during the monitoring of 8 ESS (Makiso-Kisangani, Kabondo and Tshopo) and initial assessment of the PEKIS Health Center (score: 24%); screening and hand hygiene of approximately 250 participants at the RIA; cumulatively, 59 out of 107 ESS (55.1%) received IPC kits; 5 EDS carried out and 1 body swabé.
- **Haut-Uélé:** 1 ring open out of 1 (100%) in Pawa, 6 households decontaminated out of 7 (85.7%; 5 in Pawa and 1 in Rungu) and 1 out of 1 ESS in Pawa; 2 ESS assessed in Pawa (PS Umoja: 21%; PS Providence: 5%), 14 providers briefed (1 doctor and 5 hygienists in Isiro, 5 nurses in Wamba and 3 in Pawa) on standard precautions and DLM, 3 ESS in Wamba equipped with IPC kits and equipped with functional triage units; 3 EDS carried out out of 4 alerts (75.0%) and 3 bodies swabed.
- **Bas-Uélé:** decontamination of 1 out of 1 FOSA; monitoring and support of 11 ESS in the health zones of Buta (6), Ganga (3) and Viadana (2) and briefing of 23 service providers and 1 hygiene specialist (100%) on the wearing of PPE, waste management, sorting, the

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hand washing and preparation of chlorinated solutions; no functional triage in the structures visited and no EDS carried out.

2.4.2. Challenges.

• **In North Kivu,** none of the 12 decontaminated households received a kit (0/12), 4 rings remain closed, mainly in Oicha and Mabalako, and 48 EDS (Emergency Disinfection Services) were postponed to the following day, including 38 in Beni. **In Ituri,** 6 rings remain closed, 13 swab failures were recorded (16.7%), IPC (Infection Prevention and Control) and EDS teams remain insufficient in Mangala, Mandima, Damas, and Mambasa, and IPC supplies are lacking in Mandima, Nia-Nia, and Mambasa. **In Bas-Uélé,** only 130 out of 632 service providers (20.6%) are trained in IPC-WASH, 16 out of 20 priority health facilities (HHFs) lack functional triage, 17 out of 20 lack water for 72 hours, and no WASH partners support activities around cases. **In Haut-Uélé,** only 5 out of 7 health zones (HZs) have submitted their data, IPC supervisors are not activated in Rungu, Dungu, Boma Mangbetu, and Gombari, IPC kits are lacking for HHFs, and family resistance prevented a health data collection (HDC) in Isiro. **In Tshopo,** only 7 out of 107 HHFs (6.5%) have an IPC score above 80%, no community health workers (CHWs) or traditional healers have yet been trained in IPC, and access to water remains insufficient in HHFs.

2.5. Holistic care

2.5.1. Main actions

• **In Ituri,** 88 new admissions (1 confirmed and 87 suspected) were recorded against 101 discharges, including 12 recoveries.

Thus, 385 patients (135 confirmed and 250 suspected) are hospitalized, representing an occupancy rate of 37%; 363 out of 374 patients (97.1%) received hot meals and all children under LNPE/SLM were taken care of (11/11 at the CTE, 4/4 in daycare and 51/51 in the community).

In North Kivu, 60 new admissions (14 confirmed and 46 suspected) were recorded, compared to 65 discharges, including 8 recoveries and 2 discharges without medical advice in Mabalako. A total of 501 patients (173 confirmed and 328 suspected, including 70 children under 5) are receiving care, 143 of whom are outside of Ebola Treatment Centers (ETCs), for an ETC/TC occupancy rate of 87.3% (358/410); 11 out of 16 ambulances (68.8%) completed 22 missions, all successful, and all 25 separated infants are receiving SLM (100%).

• **In Tshopo,** 3 new admissions of suspected cases were recorded against 3 discharges; 15 patients (12 confirmed and 3 suspected) are hospitalized at the CTE Météo (occupancy rate: 41.7%; 15/36) and all received hot meals, as did the 51 PPLs being monitored.

In Haut-Uélé, 7 new admissions (3 confirmed and 4 suspected, including a pregnant woman at the Pawa Ebola Treatment Center) were recorded, compared to 3 discharges (1 recovered in Wamba and 2 non-cases). 29 patients (16 confirmed and 13 suspected) are hospitalized (occupancy rate: 17.9%; 29/162) and received hot meals, as did 42 caregivers and 77 people with disabilities. In **Bas-Uélé,** 8 new suspected cases were isolated in the health centers (Ango, Bondo, Dakwa, Monga, and Poko), and 6 discharges were recorded in Titule after clinical improvement, bringing the number of isolated suspected cases to 50 due to the lack of a functioning Ebola Treatment Center.

2.5.2. Challenges

In Ituri, the blood shortage persists, 39 patients are critical, 11 patients did not receive nutrition in Mandima, capacity remains low in Mambasa, Lolwa, Mandima, and Biakato, and the Kasenyi Ebola Treatment Center (ETC) did not report due to a strike by healthcare providers (completeness: 28/29). In North Kivu, ETCs/CTs remain close to saturation (87.3%), 143 patients are being treated outside ETCs/CTs, 21 patients are critical, and the completeness of patient care data is only 71%; in Tshopo, a building at the Bafwasende ETC was burned down, 2 confirmed escapees remain missing, and patient care data has not been entered into the DHIS2 system. In Haut-Uélé, staff demotivation, insufficient nutritional inputs (F75, F100, Plumpy'Nut) and poor network coverage of CTEs persist; in Bas-Uélé, the standard CTE in Buta is still under construction and none of the 50 isolated suspects are receiving nutritional support.

2.6. Vaccination

2.6.1. Main actions

In Ituri, vaccination of people living with HIV (PLHIV) using Ervebo and research on its effectiveness against BBDV-EV continue: 233 people were vaccinated during the day (111 women and 122 men, including 2 pregnant women and 5 breastfeeding women), bringing the total to 2,448 (1,036 women and 1,412 men), with no adverse events following immunization (AEFIs) reported during the day. **In North Kivu,**

200 PPLs have been vaccinated in the Beni health zone, bringing the total to 1,000 out of 7,105 expected (14.1%), with 1,600 doses remaining out of 2,600 received, and the ultra cold chain has been installed and activated with the support of MSF France.

2.6.2. Challenges

In Bas-Uélé, the start-up of the UCCs is still not effective and the support of field actors is lagging behind; in North Kivu, the coverage of the PPLs in Beni remains low (14.1%) and 1 non-serious AEFI has been notified since the start of vaccination; in Tshopo, the activation of the continuity plan of the PEV is still awaited.

2.7. Continuity of Care

2.7.1. Main Actions • In North Kivu, the

baseline assessment of the Mukuna Hospital and the Matanda Secondary Hospital revealed triage that did not follow the established protocol, insufficient PPE and poor waste management, the absence of an emergency department, an unsuitable neonatal unit (no incubator or oxygen concentrator), no providers trained in ITP and HIV, non-integrated nutritional care, and the absence of a contingency plan for absenteeism, despite 100% of staff being trained in IPC; **in Tshopo,** the recovery plans for the La Promédie Medical Center (Mangobo), the Tshopo 1 Health Center, and the Maternity Medical Center without

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risk (Makiso-Kisangani) are being finalized, and compliance with the free provision of essential generic medicines provided by the UG-PDSS is being monitored in the ESS.

2.7.2. Challenges

Insufficient labile blood products in Ituri and at the Matanda secondary hospital (North Kivu); absence of a dashboard to monitor service disruptions and weekly supervisions in the assessed health establishments in North Kivu; implementation of recovery plans, CSSE assessments not carried out in the Lubunga health zone and decline in the use of curative, maternity, IMCI and vaccination services in Tshopo; no data transmitted by Haut-Uélé, Bas-Uélé and Sud-Ubangi.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri,** 7,910 home visits reached 47,161 people, 849 educational talks brought together 9,342 people, 85 mass awareness campaigns reached 3,374 people and 8 community debates reached 379 people; 295 community alerts, including 118 deaths, were notified, 29 out of 30 feedbacks (96.7%) and 10 out of 11 incidents (90.9%) were resolved within 48 hours; the CREC facilitated the pre-listing of 1,084 contacts around 13 confirmed cases, the decontamination of 53 out of 53 households, the completion of 19 out of 20 EDS and the transfer of 74 out of 80 suspects (92.5%); 30 RECOs from the AS of Pekele (Lolwa) were trained with the support of MEDAIR, 16 local leaders from the village of Ngombenyama (Bunia) engaged through community dialogue, 3 advocacy efforts conducted with religious authorities and 28 spots broadcast on 36 radio stations, with 10 interactive programs.

- In North Kivu, 14,921 home visits reached 51,133 people, 565 educational talks reached 11,292 people, 141 mass communication sessions reached 9,144 people and 15 briefings reached 598 people; 510 out of 650 alerts (78.5%) were notified, 34 out of 43 feedbacks (79.1%) and 8 out of 10 incidents (80.0%) were resolved within 48 hours and 2,703 out of 4,750 people (56.9%) were engaged in community actions; 30 RECOs and community actors visited the CTE in Matanda (Katwa) and 6 community members visited the one in Kitatumba (Butembo) to counter rumors about care, 30 taxi drivers were trained on CREC and EDS in Musienene, leaders and officials of armed groups in the Kambau group were sensitized in Manguredjipa, and in Beni, advocacy with the base chiefs of Benengule II allowed the decontamination of a resistant household in Rwangoma.

- In Haut-Uélé, 148 home visits reached 1,246 people, 35 educational talks reached 385 people and 7 mass activities in the schools of Isiro and Wamba reached 4,003 people; 13 community alerts and 4 death alerts were notified, 65 people were sensitized around 4 community deaths to facilitate contact tracing, decontamination and EDS, CREC supervised the integration of Ebola messages into the distribution of mosquito nets (AS N'Sele, Gossamu and Mayogo) and Samaritan's deployed communication equipment in Wondote and Mangese and sensitized motorcyclists in Dungu.

- **In Tshopo,** for the days of October 30 and 31, 497 home visits reached 2,752 people, 11 mass awareness campaigns reached 14,538 people, including 958 students from the Sainte Vierge Marie school complex (Kabondo), and 5 educational talks reached 251 people, in addition to 3 talks with the inhabitants of the Wagenia Falls on the raising of alerts; 6 interactive programs were held, 63 posters and 186 advice cards were distributed, 2 out of 3 refusals were lifted and 3 out of 3 feedback issues were resolved, and the provincial CREC actors and those of the 7 ZS hotspots were trained in infodemic management.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

In Ituri, 15 out of 28 affected health zones (53.6%) were covered, 27 out of 34 cases were confirmed (79.4%), 88 out of 112 cases were confirmed, and 133 out of 163 suspected cases were under follow-up. 57 out of 87 new suspected cases were treated. 29 out of 32 households with separated and orphaned children (90.6%) received support, 171 children were monitored, including 15 in daycare, 7 out of 12 were reintegrated, and 43 results were announced, including 27 positive results. 622 caregivers and 95 people living with HIV received support.

- In North **Kivu,** 7 out of 9 health zones (77.8%) were covered, 20 out of 25 new cases (80.0%), 296 confirmed cases under follow-up, 98 new suspected cases and 245 suspected cases under follow-up were taken care of, all 40 households of separated and orphaned children (100%) were accompanied, 28 out of 36 children (77.8%) were followed up in the community, 10 recovered patients were reintegrated, 93 results were announced, including 27 positive results, and 517 people benefited from psychoeducation (UNICEF support).

- In Haut-Uélé, 2 new confirmed cases, 16 confirmed and 13 suspected cases under follow-up as well as 12 new suspected cases were taken care of in Isiro, Pawa, Boma Mangbetu and Wamba, 12 results were announced (2 positive and 6 negative), 15 separated children are being followed in the community and 42 caregivers and 44 PPLs were supported.

- In Tshopo, 10 confirmed cases under follow-up at the CTE Météo received psychological support and 6 suspected cases received psychoeducation, 1 negative result was announced, 3 recovered patients were reintegrated in Kabondo (3/3) and psychoeducation on barrier gestures was carried out at Lycée Mapendano (185 students), at EP Alibuku and with accompanying persons and PPLs.

- **In Bas-Uélé,** no SMSPS interventions were reported for the day, apart from the follow-up of a recovered patient. unloaded.

2.9.2. Challenges

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In Ituri, only 15 out of 28 affected health zones (53.6%) are covered, and coverage remains incomplete around the most recent cases in Bunia (6/9).

In North Kivu, SMSPS coverage remains low around the most recent cases in Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi, and no childcare facilities are available in 10 affected health zones. In Haut-Uélé, no childcare facilities are available for separated children in Isiro, Pawa, Wamba, and Boma Mangbetu, and community health workers lack transportation. In Tshopo, community health workers in the Wanie-Rukula health zone are not trained, and the health unit lacks communication and transportation resources. In Bas-Uélé, psychosocial support for the three confirmed cases is delayed.

2.10. Logistics

2.10.1. Main actions

• **In North Kivu,** furniture and IPC and EDS inputs were received from WHO, 4 ESS in Katwa and Kyondo were equipped with IPC kits and the ultra cold chain for vaccine storage was installed and activated with the support of MSF France; for health products and medicines (HPM), the Beni laboratory reports needs for RNA extraction kits and Altona 1.0/2.0 PCR kits, printer cartridges and sample racks.

• **In Tshopo,** the Kisangani laboratory is in pre-breakdown of MVE buffer and two modules of the Radione platform are out of order, the faulty solar system regulator interrupting the power supply in the evening, while cumulatively 59 out of 107 ESS (55.1%) have been provided with PCI kits and compliance with the free provision of essential generic medicines provided by the UG-PDSS is being monitored in the ESS.

• **In Haut-Uélé,** 3 ESS in Wamba were provided with PCI kits and the coordination recommended making available to the ZS the stock and monitoring sheets for the use of inputs, while the PEC pillar reports a shortage of nutritional inputs (F75, F100 and Plumpy'Nut); **in Bas-Uélé,** the construction of the CTE at the HGR in Buta continues, but the 20 priority ESS do not have mattresses and 17 out of 20 lack water storage tanks.

2.11. Security

2.11.1. Main actions

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In Tshopo, part of the Bafwasende Ebola Treatment Center (ETC) was set on fire by youths from pressure groups, and increased security at the site was recommended; in North Kivu, the IPC team was accompanied and secured for the decontamination of a household in the Bioken health district; in Bas-Uélé, the security situation related to the strike was assessed, the Point of Control (PoC) at Zega Airport in Buta was visited, and a community death alert was raised by the
CIAT urban area of Buta; in Haut-Uélé, no data has been transmitted by the security pillar in the last 24 hours
hours.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

In North Kivu, 26 WHO staff (including 4 women) were briefed on the PSEA in Butembo, 22 registered nurses and 76 RECOs in Musienene, and 1,294 people were sensitized on reporting via 495555/151 in Katwa (992), Butembo (243), Beni (39, including 16 breastfeeding women) and Kyondo (20); in Tshopo, 200 participants at the RIA in Kisangani were briefed on the code of ethics and good conduct, of which 190 (95.0%) signed it; in Ituri, 24 workers from repair and carpentry workshops were sensitized in the Bora Uzima and Mudzimaria health areas (Bunia health zone). Nationally, 1,298 people were made aware of the issue during the day, bringing the total to 80,500.

2.12.2. Challenges

• Coverage of affected areas remains low: only 56 out of 151 health zones (37.1%) conduct PSEA activities, and only one Partners out of 16 (6.3%) report their activities; the reporting mechanism is known in only 16 out of 36 health zones (44.4%) in Ituri, 8 out of 34 health zones (23.5%) in North Kivu and 7 out of 23 health zones (30.4%) in Tshopo; visibility supports remain insufficient for mass awareness campaigns, CBCMs lack effectiveness and 10 participants in the Kisangani RIA refused to sign the code of good conduct.

KEY MESSAGES

No new health zones have been affected in the last 24 hours.

67 out of 178 health zones (37.6%) remain affected in eight provinces.

Fifty-three (53) new confirmed cases were reported in three provinces, including 32 in Ituri, 18 in North Kivu and 3 in Haut-Uélé. Transmission remains particularly marked in the health zones of Beni (13 cases), Mandima (9 cases), Bunia (8 cases) and Mambasa (5 cases).

Of the 45 confirmed deaths reported, 26 are community deaths (13 in Ituri, 12 in North Kivu and 1 in Haut-Uélé) and 19 are deaths at Ebola Treatment Centers: 15 in Ituri and 4 in North Kivu. The contact tracing rate stands at 86.6% (31,621/36,517). up compared to the previous day (85.8%) but still below the 95% threshold,

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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